Williams Physical Therapy

93183 Lisa Fort
Baytown, TX
Phone 281-555-9042

Itemized Medical Bill

Patient:	Scott Perry
Date of birth:	April 18, 1992
Insurance claim:	C-1273
Date of injury:	December 28, 2024
First visit:	December 31, 2024
Last visit:	May 15, 2025
Number of visits:	23

Services

Service	Units	Charge
99203 New patient evaluation	1	$281.61
98941 Spinal manipulation	5	$500.13
97110 Therapeutic exercise	2	$328.30
97124 Massage therapy	4	$384.25
72040 X-ray, cervical spine	4	$2,846.67
20552 Trigger point injection	7	$3,781.07
TOTAL		$8,122.03

Recent visits

Date	Service	Charge
April 8, 2025	97124 Massage therapy	$104.22
April 12, 2025	97124 Massage therapy	$107.45
April 22, 2025	98941 Spinal manipulation	$81.27
April 30, 2025	97124 Massage therapy	$91.95
May 9, 2025	98941 Spinal manipulation	$126.41
May 15, 2025	97110 Therapeutic exercise	$188.03

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Treatment History and Summary

Patient:	Scott Perry
Treating provider:	Tommy Griffin, DC
Diagnosis:	knee injury

Treatment history by month

Month and services	Visits	Charges
December 2024: evaluation	1	$281.61
January 2025: imaging, pain management, physical therapy	4	$1,719.56
February 2025: imaging, massage therapy, pain management	5	$2,804.88
March 2025: chiropractic adjustment, imaging, pain management	7	$2,616.65
April 2025: chiropractic adjustment, massage therapy	4	$384.89
May 2025: chiropractic adjustment, physical therapy	2	$314.44

Treatment summary

Patient presented following a rear-end motor vehicle collision with complaints of right knee pain, swelling, and reduced range of motion. The mechanism of injury described is consistent with the knee findings observed on examination. Imaging and physical examination clearly confirm the injury, and the findings are consistent with the reported mechanism. The patient has a remote history of a minor strain involving the same region, which resolved completely without residual limitation and did not affect the current presentation. Over a course of 23 visits, care progressed through a conventional sequence of therapeutic exercise, manual therapy, and activity modification. Symptoms, strength, and mobility improved steadily, and the frequency of visits tapered as the patient recovered. Billing recodrs reflect single, appropriately spaced charges for each service rendered, with no repeated billing. The patient is currently near maximum medical improvement, with residual stiffness expected to continue to resolve with a home exercise program.